INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Ceritinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Treatment pattern**
**Ongoing**
Follow the prescribed daily plan

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00340**
Version 4

> Once daily with food; no rest week

## Treatment stages

01 **Continuous oral ceritinib**
Daily with food

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

## How this medicine supports your treatment

Ceritinib blocks abnormal ALK signals that help some lung cancers grow. In advanced cancer after crizotinib, the aim is to control cancer and delay progression. The choice depends on your tumour test, previous treatment, other options and priorities.

ALK TARGETED THERAPY

### Ceritinib

**Blocks abnormal ALK growth signalling**

ALK-positive tumour confirmed → Growth signal blocked → Cancer growth slowed

Ceritinib inhibits ALK signalling in a tumour with a confirmed ALK rearrangement. It can affect healthy tissues too, so blood tests and symptom checks continue even when treatment is taken at home.

Report breathing changes, fainting, jaundice, severe abdominal pain, persistent diarrhoea/vomiting or marked thirst promptly.

## Your treatment and its purpose

### Confirm the setting

Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 v4 · official regimen

### Duration and review

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

NCCP 00340 v4 · official regimen · EMA Zykadia product information, retrieved 29 October 2026

## Evidence after crizotinib and for dosing with food

Study results describe groups, not an individual forecast.

Trials used different doses and populations. No proven individual benefit or cross-trial ranking is implied.

## When to get help

**Emergency: 112 / 999.** Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

**Contact your treating team promptly:** Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your agreed out-of-hours pathway**

Other hospitals: use the contacts your own team provides.

CHAPTER 02

# Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

# Ceritinib checks over time

Columns are milestones, not equal durations. The team books actual tests and reviews; no patient-entered dates are needed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00340 v4 · official regimen · EMA Zykadia product information, retrieved 29 October 2026

THE WHOLE TREATMENT JOURNEY

# Ceritinib monotherapy: treatment and checks

**Your treatment pattern**

Once daily with food; no rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

## ALK-positive advanced non-small-cell lung cancer (NSCLC) · after crizotinib

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First 3 months | After month 3 | Benefit review | Hold / change / stop |
|---|---|---|---|---|---|
| **Daily ceritinib** NCCP 00340 · SMPC | Confirm ALK, previous crizotinib and prescription | Daily with food; no planned rest week | Continue authorised daily schedule | Assess benefit and tolerability | Written instructions for any change |
| **Liver tests** NCCP 00340 · SMPC | alanine aminotransferase (ALT), aspartate aminotransferase (AST) and bilirubin | Every 2 weeks | Monthly; sooner if abnormal | Continue tests and symptom review | Check recovery before any restart |
| **Blood count, kidneys and glucose** NCCP 00340 · SMPC | full blood count (FBC), renal profile and fasting glucose | Every 4 weeks; glucose sooner if needed | Every 4 weeks and when indicated | Review diabetes, symptoms and medicines | Follow unresolved abnormalities |
| **Heart and pancreatic checks** NCCP 00340 · SMPC | electrocardiogram (ECG), pulse/blood pressure (BP), lipase and amylase | ECG after 1 month; other checks as needed | ECG/ electrolytes and pancreatic enzymes as indicated | Review pulse/ BP and symptoms | Investigate fainting or abdominal pain |

| Care / check | Before starting | First 3 months | After month 3 | Benefit review | Hold / change / stop |
|---|---|---|---|---|---|
| **Symptoms, interactions and supply** NCCP 00340 · SMPC | Review medicines, food and contact plan | Report breathing/ gastrointestinal (GI) symptoms promptly | Check adherence, prescriptions and supply | Scans and disease assessment per consultant | Confirm follow-up and who authorises restart |

## What each check is for

### 1 Safe start

Before first dose

Correct biomarker, treatment line and safety context matter.

**Ask / act:** Confirm dose, food instructions, support medicines and contacts.

### 2 Early liver surveillance

Every 2 weeks for 3 months

Liver injury may occur without early symptoms.

**Ask / act:** Confirm test bookings and result reviewer.

### 3 Regular clinical checks

Throughout daily treatment

Continuous tablets still need continuing review.

**Ask / act:** Review symptoms, interactions, intake, supply and scans.

### 4 Dose held or treatment stopped

Toxicity, progression or changed priorities

Restart criteria differ by toxicity and food schedule.

**Ask / act:** Prescriber gives a new written plan and follow-up.

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

# Symptoms and supportive care

**IN THIS CHAPTER**

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

### Daily treatment, tests and next decisions

No personal dates are needed. Confirm actual appointments with your team.

| Treatment or check | Timing in this view |
|---|---|
| **Preparation** | **Before starting**<br>Confirm tumour test, prior treatment, baseline checks and medicines |
| **Daily capsules** | **First 3 months → After month 3 → Benefit review**<br>Take the prescribed dose with food every day |
| **Blood tests and reviews** | **First 3 months → After month 3 → Benefit review → Hold / change / stop**<br>Fortnightly early liver tests; regular blood, glucose and heart checks |
| **Next plan** | **Hold / change / stop**<br>Clinician authorises continuation, hold, change or stop |

Do not alternate fed and fasting dosing.

Columns are milestones, not equal time periods.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

## Taking ceritinib at home

Food changes absorption, so the dose and food instructions belong together.

### Daily, missed or vomited doses

Take at the same time daily with food. Swallow capsules whole with water; do not chew or crush. Take a missed dose unless the next is due within 12 hours; ask your pharmacist if timing is unclear. Never double a dose. If you vomit, do not replace the dose; take the next at its usual time.

### If you cannot eat

If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

## Tests and reviews

Baseline tests and ongoing checks do different jobs.

### Your monitoring plan

Before starting: ALK test, blood count, kidney and liver tests, pancreatic enzymes, fasting glucose, ECG and symptom/medicine review. Blood count, kidneys and fasting glucose every 4 weeks; liver tests every 2 weeks for the first 3 months, then monthly. ECG after 1 month, then as needed. Extra heart, electrolyte, glucose or pancreatic tests may be needed for risks, symptoms or abnormal results; scans follow the consultant plan.

### Symptoms can bring a review forward

Report new symptoms between visits. Tests alone cannot establish whether symptoms are caused by treatment.

## Support during treatment

Ask for help with symptoms, daily routines and practical concerns.

### Prevent and treat nausea

Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.

NCCP 00340 v4 · official regimen · NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), Tables 5 and 8 · NCCP supportive antiemetics for NCIS Medical Oncology; check local policy

### Check interactions

Avoid grapefruit or grapefruit juice and St John’s wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.

### Pregnancy, fertility and feeding

Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.

### Diarrhoea, fluids and daily life

Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

## Symptoms needing prompt help

Keep the oncology number available day and night.

### Do not wait for the next visit

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

### When you call

Tell the team the medicine, last dose and new symptoms; do not delay calling to collect details.

## Benefit and choices

A study after crizotinib and platinum chemotherapy found longer disease control with ceritinib than the chemotherapy used at that time. It tested an older fasting dose. A separate dose study supports taking the current lower dose with food; its main efficacy group had not had previous treatment. These studies cannot predict your outcome or rank all current treatment options.

### Match evidence to your situation

ASCEND-5 involved previous crizotinib and platinum chemotherapy and an older fasting dose. ASCEND-8 supports the fed schedule but its main efficacy analysis was treatment-naive. Ask how these results fit your previous treatments and current alternatives.

ASCEND-5: post-chemotherapy/crizotinib phase III trial, Shaw 2017 · ASCEND-8: pharmacokinetic and food-effect study, Cho 2017 · ASCEND-8: primary efficacy and safety analysis, Cho 2019

### Review your choices

The team reviews symptoms, scans, side effects and priorities. Discuss alternatives, supportive care and choosing not to proceed; you may reconsider consent.

## Practical support

### Prevent and treat nausea

### Food and daily doses

### Check interactions

### Pregnancy, fertility and feeding

### Diarrhoea, fluids and daily life

Taking tablets / attending treatment

**Practical treatment details**

### Refills and interruptions

Request repeats early.

## Questions to ask

**Do I need to enter dates?**

No. Your team books actual appointments; this map shows general milestones.

**Explain the plan back:** Can you explain taking the prescribed dose with food, the 12-hour missed-dose instruction, what to do if you cannot eat, your next liver test and when to call urgently?

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**Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib** Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Ceritinib daily oral treatment

**Active medicines:** ceritinib

**Emetogenic risk:** Moderate to high (oral)

| Medicine | Dose / route | When |
|---|---|---|
| Individually selected daily oral antiemetic prophylaxis | Prescriber/pharmacy to select and prescribe under local oral-systemic anticancer therapy (SACT) policy | Daily while required during oral treatment; review prevention and rescue needs<br>NCCP V6 Table 5 supports daily oral 5-HT3 antagonist prophylaxis for moderate-high risk. Ceritinib can prolong QT and cause bradycardia; check electrocardiogram (ECG)/electrolytes and interactions before selecting an agent. Do not copy an infusion steroid/NK1 bundle. |

## Additional phase-specific support

- Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.
- If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.
- Avoid grapefruit or grapefruit juice and St John’s wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.
- Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.
- Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

**Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib** Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: National Cancer Control Programme (NCCP) 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

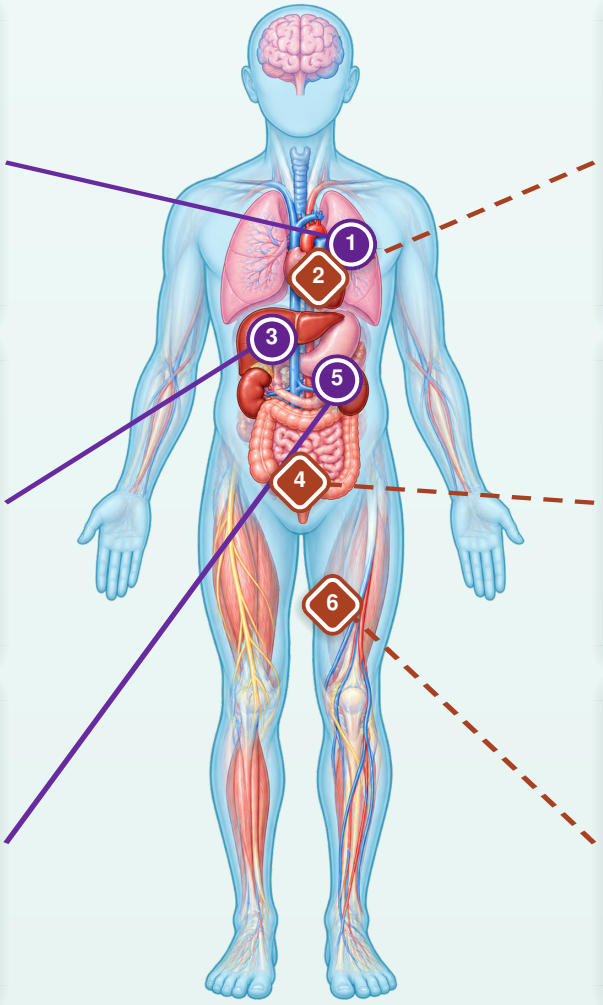
# Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



**1 New cough or breathlessness**

ceritinib

Possible lung inflammation needs urgent assessment.

**2 Fainting or palpitations**

ceritinib

Slow pulse and rhythm changes can be serious.

**3 Yellow skin or dark urine**

ceritinib

Blood tests detect liver changes; symptoms need urgent advice.

**4 Diarrhoea or vomiting**

ceritinib

Persistent symptoms can cause dehydration.

**5 Persistent abdominal pain**

ceritinib

Pancreatic inflammation needs prompt assessment.

**6 Thirst and frequent urination**

ceritinib

High glucose may need urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00340 v4 · pNCCP 00340 v4 p5; classification Tables 5/8; current SmPC interactions**
   https://healthservice.hse.ie/documents/6917/340_Ceritinib_monotherapy.pdf
2. **EMA Zykadia product information, retrieved 29 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/zykadia-epar-product-information_en.pdf
3. **NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), Tables 5 and 8**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
4. **NCCP supportive antiemetics for NCIS Medical Oncology; check local policy**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **ASCEND-5: post-chemotherapy/crizotinib phase III trial, Shaw 2017**
   https://pubmed.ncbi.nlm.nih.gov/28602779/
6. **ASCEND-8: pharmacokinetic and food-effect study, Cho 2017**
   https://pubmed.ncbi.nlm.nih.gov/28729021/
7. **ASCEND-8: primary efficacy and safety analysis, Cho 2019**
   https://pubmed.ncbi.nlm.nih.gov/30851442/